

CARDIOLOGY PROCEDURE NOTE

[SAMPLE - FOR TESTING PURPOSES ONLY]

PATIENT INFORMATION

- **Name:** John A. Doe
 - **DOB:** 03/15/1965
 - **MRN:** 123456789
 - **Date of Procedure:** August 28, 2025
 - **Provider:** Dr. Sarah Johnson, MD, FACC
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PROCEDURE PERFORMED Atrial Septectomy - Open Heart with Cardiopulmonary Bypass

INDICATION 18-month-old patient with transposition of the great arteries (TGA) and inadequate atrial mixing causing severe cyanosis and failure to thrive. Previous balloon atrial septostomy inadequate.

PROCEDURE DETAILS The patient was brought to the operating room and placed under general anesthesia with endotracheal intubation. Standard monitoring lines were placed including arterial line, central venous catheter, and urinary catheter.

A median sternotomy was performed with careful dissection due to previous cardiac procedures. Cardiopulmonary bypass was initiated using aortic and bicaval cannulation. The patient was cooled to 28°C. Cardioplegic arrest was achieved using cold blood cardioplegia.

The right atrium was opened, and the atrial septum was visualized. The existing septostomy site was identified and found to be restrictive. Using electrocautery and surgical scissors, the atrial septum was excised from the superior vena cava to the inferior vena cava, creating a large atrial communication.

Hemostasis was achieved, and the right atrium was closed in two layers using 5-0 Prolene suture. The patient was rewarmed, and the heart resumed normal sinus rhythm spontaneously.

FINDINGS

- Restrictive atrial septum with previous balloon septostomy site
- Adequate atrial septectomy created measuring approximately 15mm
- Good hemostasis achieved
- Normal cardiac function post-procedure

- Improved oxygen saturation from 75% to 85%

ASSESSMENT & PLAN Successful atrial septectomy with improved atrial mixing and oxygen saturation. Patient is now a better candidate for future arterial switch operation. Plan includes:

1. ICU monitoring for 24-48 hours
2. Gradual weaning from mechanical ventilation
3. Continue prostaglandin E1 infusion to maintain ductal patency
4. Anticoagulation with heparin per protocol
5. Cardiothoracic surgery follow-up in 2 weeks
6. Plan for arterial switch operation in 4-6 weeks pending clinical stability

COMPLICATIONS None

PATIENT CONDITION Stable, no acute distress. Vital signs within normal limits. Patient ambulating without difficulty.

Provider: Dr. Sarah Johnson, MD, FACC

Date: August 28, 2025

Electronically signed

This is a sample document created for testing purposes only and does not represent actual patient care.